

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Patient Name: Patient 35

MRN: 000-TEST-35

Age: 62 years old | Sex: Male

Admission Date: 06/10/2026 | Discharge Date: 06/15/2026

Admitting Physician: Dr. M. Chen, Orthopedic Surgery

****PRIMARY DIAGNOSIS & COMORBIDITIES****

- Primary: Severe left hip osteoarthritis s/p left total hip arthroplasty (THA) with cemented prosthesis
- Secondary diagnoses: COPD (GOLD Stage II), peripheral artery disease (PAD) of bilateral lower extremities, hypertension, hyperlipidemia, type 2 diabetes mellitus

****HOSPITAL COURSE****

Patient 35 was admitted on 06/10/2026 for elective left THA via posterolateral approach. Intraoperative course uncomplicated; cemented acetabular and femoral components placed without intraoperative adverse events. Estimated blood loss: 450 mL. Patient tolerated procedure well under general anesthesia with epidural analgesia.

Post-op Day (POD) 1: Pain controlled with multimodal analgesia (see medications below). Physical therapy initiated; patient ambulated to chair with walker, tolerating weight-bearing as tolerated. Foley catheter removed POD #1. O2 saturation maintained 92–96% on room air; baseline COPD stable, no acute exacerbation.

POD 2–5: Steady progress with PT/OT. Patient achieved independent ambulation with front-wheeled walker on tile, stairs negotiated with contact guard assistance. Hip precautions reinforced daily—no hip flexion >90°, no internal rotation, no hip adduction past midline. Wound healing appropriate; sutures intact, no erythema or drainage. Compression stockings applied BID for DVT prophylaxis; enoxaparin 40 mg SC q.d. initiated per protocol (see below). Patient discharged on POD #5 in good condition.

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
Enoxaparin (Lovenox)	40 mg	SC daily	14 days post-discharge
Oxycodone-acetaminophen	5–10 mg PO	Q4H PRN pain	2 weeks
Ibuprofen	600 mg	TID with meals	4 weeks
Docusate (Colace)	100 mg	BID	ongoing until bowel regimen established
Omeprazole	20 mg	q.d.	ongoing
Lisinopril	10 mg	q.d.	ongoing
Atorvastatin	40 mg	q.d. at bedtime	ongoing
Albuterol inhaler	2 puffs	Q4–6H PRN dyspnea	ongoing
Tiotropium (Spiriva)	18 mcg	inhaled daily	ongoing

****ACTIVITY & WEIGHT-BEARING STATUS****

- Weight-bearing as tolerated (WBAT) on left lower extremity
- Use front-wheeled walker for all ambulation until cleared by PT (approx. 4–6 weeks)
- NO stairs without rail and supervision for first 2 weeks
- Sleep with pillow between knees × 6 weeks to maintain hip abduction
- Avoid prolonged sitting; use elevated toilet seat and firm chair (seat height ≥18 inches)

****HIP PRECAUTIONS (CRITICAL)****

- NO hip flexion beyond 90 degrees
- NO internal rotation of